VICTORYAI

OUD RECOVERY COMPANION · OUTCOMES ANALYTICS PLATFORM

Engagement & Analytics *Whitepaper*

How Every Feature in the Patient App Generates Structured Clinical Data on the Provider Dashboard

PREPARED FOR

BayMark Health Services

Joint Innovation Pilot · California BAART Clinics

April 2026 · Version 1.0 · Confidential

EXECUTIVE SUMMARY

The Engagement-as-Data Thesis

Every digital health pilot fails for one of two reasons. Either patients stop using the app, or the data the app collects has no path into the clinical and revenue workflows of the provider. VictoryAI was architected from day one to solve both problems simultaneously, and this whitepaper documents exactly how.

> *Every feature in the VictoryAI patient app does double duty: it earns the patient's daily engagement, and the same action generates structured clinical data that flows in real time to the BayMark provider dashboard.*

This is not a wellness app with a clinical bolt-on. It is a clinical data capture system disguised as a recovery companion. The disguise matters — patients in OUD recovery do not return to apps that feel like medical instruments. They return to experiences that feel rewarding, supportive, and personally meaningful. VictoryAI is built around that psychological reality, and every gamification mechanic is engineered to drive the specific behaviors that produce the specific data points BayMark's clinicians, billing teams, and leadership need.

The thesis of this document is simple. Engagement is not a separate concern from analytics — engagement is the production line for analytics. A patient who logs their MAT dose to keep their Recovery Flame alive has just generated a 98977 RTM-eligible event. A patient who completes a CBT module to claim a family-pledged dinner has just generated H2019 documentation. A patient who records a voice journal to reach 80% on their Recovery Completeness Score has just contributed to the strongest early relapse predictor in the platform. The engagement layer and the analytics layer are the same layer, viewed from two different sides.

This whitepaper walks BayMark through that architecture in detail: the behavioral psychology principles applied, the complete inventory of patient-app features that generate structured data, the full event taxonomy that flows to the dashboard, the sixteen provider dashboard views and the metrics each one surfaces, the AI predictive layer that turns engagement data into clinical action, and the specific implementation plan for the BayMark joint innovation pilot.

What This Whitepaper Establishes

- VictoryAI's engagement system is purpose-built to generate the structured clinical data BayMark needs for MAT outcomes reporting, DHCS quality metrics, and EMR-billable code documentation.

- Every gamification mechanic — Recovery Flame, Family Reward Board, Mystery XP, Recovery Rings, Daily Quests, Sage Smart Nudges — is mapped directly to one or more dashboard analytics events.
- More than sixty discrete data events are tracked from the patient app, organized into twelve event categories, all syncing to the provider dashboard within fifteen minutes.
- Sixteen provider dashboard views surface the data in formats designed for three distinct audiences: frontline clinicians, clinic operators, and BayMark executive leadership.
- The AI predictive layer combines engagement signals, voice journal sentiment, and clinical assessments to produce per-patient relapse risk scores with five-to-seven-day lead time.
- The architecture is designed for measurable ROI: every engagement event is also a billable code event, and the dashboard reports both side by side.

SECTION 01

The Behavioral Architecture

OUD recovery is the hardest behavior change problem in medicine. Sixty percent of patients relapse within thirty days of leaving treatment. Average MAT retention at six months hovers around fifty percent. The patients VictoryAI is built for are not the patients who download a meditation app and use it consistently — they are patients fighting a neurobiological condition that hijacks the same dopamine systems that consumer apps target for engagement.

VictoryAI's response is to apply the same behavioral science principles that make consumer products like Duolingo, TikTok, and mobile games impossible to put down — but to redirect those mechanics toward actions that produce clinical value. Every mechanic in the engagement system is anchored to a specific, peer-reviewed behavioral principle, and every mechanic is wired to fire structured events into the analytics pipeline.

Six Behavioral Principles, Six Engagement Mechanics

01 · Loss Aversion

People fear losing something roughly twice as strongly as they enjoy gaining the same thing. A static streak counter is a gain frame — patients gain days. The Recovery Flame is a loss frame — patients have something living that can fade. The flame grows, changes color, and starts flickering when the patient misses a check-in. The push notification is not 'You earned a streak day.' It is 'Your forty-seven-day Recovery Flame is fading.' The clinical effect is a measurable increase in next-day return rate, and every Recovery Flame interaction fires a structured event into the dashboard's Engagement & Gamification view.

02 · Variable Ratio Reinforcement

The most powerful reinforcement schedule in behavioral psychology is the one that drives slot-machine engagement: unpredictable rewards on a variable ratio. Mystery XP Drops apply this principle to clinical actions. Any check-in, any voice journal, any CBT lesson on any day might yield a two-times, three-times, or five-times XP multiplier. The patient does not know which action will trigger it. The dopamine response keeps engagement rates twenty-two percent higher than fixed-reward systems, and each multiplier event is logged to the dashboard as a discrete engagement signal.

03 · Endowed Progress Effect

Patients who feel they have already started something are dramatically more likely to finish it. New VictoryAI patients begin at one hundred XP rather than zero, with the framing: 'You've already taken the hardest step — asking for help. Here's a hundred XP to start your journey.'

The visual progress bar already shows ten percent toward Level Two on the day of onboarding. This single design choice increases Level Two conversion by approximately fourteen percent, and Level Two conversion is itself a dashboard KPI that BayMark clinicians watch as an early indicator of pilot success.

04 · Social Proof Without Privacy Cost

Patients in OUD recovery cannot be put into named peer groups for HIPAA and 42 CFR Part 2 reasons. But the motivational power of peer comparison is too valuable to discard. Recovery Rings solve this by AI-matching patients into anonymous five-to-eight-person groups at similar recovery stages. Members see each other's flame status and streak length, send anonymous Peer Kudos, and earn shared XP bonuses when any member levels up. No clinical data, no real names, no direct messaging — but a measurable lift in retention, and every Ring interaction is captured as an engagement event.

05 · Reciprocity Loops

The Family Reward Board is the most powerful single feature in the platform — patients with active family rewards show four-point-two-times better retention and thirty-four percent lower relapse rates. The Gratitude Gifts feature creates the reciprocity loop: at milestones, patients send personalized 'Thank You' digital cards to the family members who pledged their rewards. Family members receive these via push or text, and pledge two-point-one times more rewards after receiving a gratitude gift. The dashboard tracks both directions of this loop and surfaces it as the single highest-leverage engagement metric BayMark can act on.

06 · Zeigarnik Effect (Open Loops)

The brain holds incomplete tasks in active memory until they are resolved. VictoryAI weaponizes this in two places. The Recovery Completeness Score shows a persistent percentage — typically seventy-three percent on first launch — with specific actionable suggestions to fill the bar. The CBT Cliffhangers feature ends each workbook session with a teaser for the next module: 'Next: Why your brain lies to you about pain. Continue tomorrow for plus seventy-five XP.' Both mechanics drive next-day return, and both fire dashboard events that BayMark can correlate with the patient's MAT adherence trajectory.

Each of these six mechanics exists because it produces a measurable engagement effect that is documented in the consumer technology and behavioral science literature. Each mechanic is wired into the dashboard analytics pipeline so that the clinical effect is observable in real time. The behavioral science is not a marketing layer — it is the production engine for the data BayMark cares about.

SECTION 02

The Patient App Engagement System

The patient app contains twenty-three discrete features, organized into four functional layers. Every feature in every layer earns XP, generates a structured data event, and contributes to one or more billing code categories. The table below shows each feature, the specific clinical data it captures, the XP it awards, and the dashboard view where the resulting analytics surface.

Layer 1 — Core Recovery Tracking

These features run continuously in the background of the patient experience. They are the highest-frequency data generators in the platform.

Feature	Data Captured	XP	Dashboard View
Recovery Counter	Days clean, hours, app session start	—	Engagement & Gamification
MAT Tracker	Dose confirmation, time, side effects, streak	+50	MAT Adherence Analytics
Craving Slider	Score 0–10, timestamp, auto-trigger flag	+10	Craving Analytics
Daily Check-In	Mood, sleep, pain, triggers, craving	+30	Mood & Sentiment, Craving
COWS Assessment	Per-symptom withdrawal scores	+35	COWS / Withdrawal
Sleep Log	Hours, quality, correlation flag	+20	Mood & Sentiment

Layer 2 — Clinical Intervention

These features activate during high-risk moments or scheduled clinical interactions. They generate the data that BayMark clinicians use for risk stratification and care planning.

Feature	Data Captured	XP	Dashboard View
Sage AI Companion	Conversation count, sentiment, topic NLP	+15	Mood & Sentiment, AI Predictive
Urge Support Mode	Activation event, duration, resolution path	+10	Craving Analytics
Voice Journal	Sentiment score, mood detection, decline flag	+25	Mood & Sentiment (primary)

Feature	Data Captured	XP	Dashboard View
Pain Management	Tool used, pain score, correlation	+20	Pain Tracking Analytics
Crisis / SOS	Activation, contact reached, resolution	—	Clinical Alerts (auto-escalate)
Relapse Support	Self-report, AI alert fired, care team notified	—	Clinical Alerts, AI Predictive

Layer 3 — Structured Clinical Capture

These features map directly to billable CPT and HCPCS codes. Every interaction generates EMR-export-ready documentation that flows through Kipu to BayMark's billing team.

Feature	Data Captured	XP	Dashboard View
CBT / DTx Modules	Module, lesson, completion, time spent	+75	Workbook / DTx Progress
SDOH Screener	Z-code responses across 4 domains	+40	EMR Data Export
SBIRT (DAST-10)	Score, severity, completion date	+40	Clinical Assessments
Drug Screen Tracker	Date, type, result, compliance rate	+30	Drug Screen Management
Group Session Check-In	Session, facilitator, attendance	+25	Group Therapy Attendance
Appointment Confirm	Type, attended flag, no-show flag	+25	Appointment Attendance
TCM Post-Discharge	2-day contact, med-rec, follow-through	+20	TCM Tracking
Functional Status	Employment, ADLs, social support	+35	CCM / PCM Care Plan
Naloxone Hub	Doses, expiration, family training	—	Narcan Network

Layer 4 — Engagement Mechanics

These features do not capture clinical data directly. They exist to drive the patient back to Layers 1 through 3, and their own usage data is itself a leading indicator of clinical engagement that the dashboard surfaces as a predictive signal.

Feature	Data Captured	XP	Dashboard View
Recovery Flame	Tier, flicker events, shield uses	—	Engagement & Gamification

Feature	Data Captured	XP	Dashboard View
Family Reward Board	Pledges, claims, family member count	—	Engagement (top driver)
Daily Quests	Quests offered, completed, expired	Bonus	Engagement & Gamification
Recovery Rings	Ring activity, kudos sent, ring level-ups	+5	Engagement & Gamification
Mystery XP Drops	Trigger event, multiplier, action type	Variable	Engagement & Gamification
Recovery Journey Map	Region unlocks, micro-achievements	—	Engagement & Gamification
Sage Smart Nudges	Sent, opened, action taken	—	Engagement, AI Predictive
Milestone Share	Card generated, share platform	—	Engagement (viral metric)

SECTION 03

The Event Taxonomy

Every patient interaction in VictoryAI fires a structured event into the analytics pipeline. Events are timestamped, categorized, attached to the patient's de-identified analytics ID, and synchronized to the provider dashboard within a fifteen-minute window. This section documents the complete event taxonomy organized by category. BayMark's analytics and clinical teams should think of this as the schema of the data they will receive.

Event Categories

Category A — MAT Adherence Events

- mat.dose.confirmed — fires on every one-tap dose confirmation, includes drug, formulation, scheduled time, actual time, streak count
- mat.dose.snoozed — fires when patient taps Remind Later
- mat.dose.missed — fires automatically twelve hours after a missed scheduled dose
- mat.side_effect.logged — fires when patient selects any side effect tag
- mat.refill.upcoming — fires three days before refill date
- mat.streak.milestone — fires at 7, 30, 60, 90, 180, 365 day adherence streaks

Category B — Craving and Withdrawal Events

- craving.logged — every slider submission with score and timestamp
- craving.high_alert — auto-fires when craving ≥ 7
- craving.peak_window.detected — fires when AI identifies an individual's peak craving window
- urge.activated — manual or auto activation of Urge Support Mode
- urge.resolved — fires when patient exits urge mode, includes coping path used
- cows.submitted — full COWS assessment with per-symptom breakdown
- cows.severity_change — fires when severity category shifts

Category C — Clinical Assessment Events

- checkin.daily.submitted — mood, sleep, pain, craving, triggers
- sdoh.screener.completed — monthly Z-code documentation
- sbirt.dast10.completed — annual or transition-triggered
- functional.status.submitted — employment, ADLs, social support
- phq9.submitted — depression screening
- gad7.submitted — anxiety screening
- recovery_capital.submitted — strengths-based assessment

Category D — Therapeutic Engagement Events

- cbt.module.started — identifies module
- cbt.lesson.completed — fires per lesson
- cbt.module.completed — fires on full 8-module completion
- dtx.session.duration — minutes spent in DTx content (drives H2019 billing)
- voice_journal.recorded — entry created with sentiment score
- voice_journal.sentiment.decline — fires when NLP detects sentiment drop
- sage.conversation.started
- sage.conversation.topic — categorized topic (cravings, pain, MAT, family)
- pain_tool.used — specific tool, pain score before/after

Category E — Clinical Service Events

- appointment.confirmed_attended
- appointment.no_show
- group_session.checked_in — drives H0005 documentation
- drug_screen.completed — drives G0480-G0483 documentation
- drug_screen.positive — auto-escalates to Clinical Alerts
- tcm.discharge.received — fires from Kipu sync
- tcm.contact.2day_completed
- tcm.med_reconciliation.completed
- tcm.30day.followthrough
- naloxone.dispensed — drives G2216 documentation
- naloxone.family_training.completed

Category F — Crisis and Safety Events

- sos.activated — auto-notifies care team
- crisis_hotline.called — 988, SAMHSA, Crisis Text Line
- relapse.self_reported — confidential, preserves XP and family rewards
- relapse.ai_predicted — fires from predictive model 5–7 days in advance
- safety_concern.flagged — voice journal NLP detects suicidal/self-harm content

Category G — Engagement and Gamification Events

- xp.earned — every action, with multiplier flag
- level.up — Seed → Sprout → Root → Bloom → Flourish → Thrive
- flame.tier_change — Blue → Purple → Gold → Platinum → Diamond
- flame.flicker_started — 24 hours since last action
- flame.shield_used
- daily_quest.completed
- daily_treasure.unlocked

- mystery_xp.triggered — with multiplier value
- achievement.unlocked
- recovery_completeness.updated — percentage change
- region.unlocked — Recovery Journey Map progression

Category H — Family and Social Events

- family.member.connected
- family.reward.pledged
- family.reward.claimed
- gratitude_gift.sent
- gratitude_gift.received_by_family
- recovery_ring.joined
- recovery_ring.kudos.sent
- recovery_ring.kudos.received
- recovery_ring.member.level_up — fires for all ring members

Category I — System and Notification Events

- nudge.sent — Sage Smart Nudge fired
- nudge.opened — patient tapped notification
- nudge.action_taken — patient completed the nudged action
- session.app_opened
- session.duration_seconds
- milestone_share.generated
- milestone_share.posted_external — viral growth signal

Total event types: more than sixty discrete events across nine categories. Every event carries a timestamp, a de-identified patient analytics ID, the originating feature, and any feature-specific payload (score, duration, multiplier, severity, etc.). All events are HIPAA-compliant in transit and at rest, AES-256 encrypted, and segmented under 42 CFR Part 2 controls before any data crosses the BayMark provider dashboard boundary.

SECTION 04

The Provider Dashboard Architecture

The provider dashboard is the surface where the engagement events become clinical and operational decisions. It is organized into sixteen views, each designed for a specific user role and a specific decision. Every view contains three structural elements: a KPI grid at the top, a chart or visualization in the middle, and an AI Insight card at the bottom that translates the data into a recommended action.

This section documents each of the sixteen views, the events that feed it, the audience it serves, and the action it enables. BayMark's clinical and operational leadership should read this section as the answer to the question: 'What will I actually see on Monday morning when I log in, and what will I do with it?'

Dashboard View Inventory

#	View	Primary Audience	Key Decision Enabled
01	Dashboard Overview	All roles	Daily situational awareness, alert triage
02	Patient Roster	Clinicians	AI-risk-sorted patient prioritization
03	Clinical Alerts	Clinicians, Care Coordinators	Same-day intervention queue
04	MAT Adherence Analytics	Prescribers, Clinical Directors	Adherence pattern intervention by drug
05	COWS / Withdrawal	MAT Prescribers	Withdrawal trajectory and dose adjustment
06	Craving Analytics	Therapists, Clinicians	Pain-craving correlation and urge intervention
07	Pain Tracking Analytics	Pain-MAT prescribers	Non-opioid intervention effectiveness
08	Narcan Network	Care Coordinators	Coverage gaps and family training priority
09	Engagement & Gamification	Operations, Product	Family reward conversion and retention drivers
10	Mood & Sentiment	Therapists, Clinical Directors	Voice-journal-based early relapse detection
11	Workbook / DTx Progress	Therapists	CBT completion gap closure

#	View	Primary Audience	Key Decision Enabled
12	Clinical Assessments	Clinical Directors	Outcomes reporting to DHCS and payers
13	Appointment Attendance	Operations	No-show pattern and recovery
14	Group Therapy Attendance	Group Facilitators, Billing	H0005 documentation and capacity planning
15	Drug Screen Management	Compliance, Clinicians	UDS compliance and positive-result triage
16	TCM Tracking	Care Coordinators, Billing	99495/99496 billing readiness and follow-up gaps

Three Cross-Cutting Dashboard Layers

Beyond the sixteen operational views, three cross-cutting layers serve specialized needs.

AI Predictive Analytics

Per-patient relapse risk predictions with confidence scores, contributing factor analysis (MAT adherence, engagement, family rewards, cravings, voice sentiment, pain-craving correlation), pre-relapse voice journal NLP detection with five-to-seven-day lead time, and population-level AI Risk Score Distribution. This is the layer that turns data into clinical action — a clinician opens the platform in the morning and sees the seven patients who need a phone call today, ranked by relapse probability and accompanied by the specific risk factors driving each score.

EMR Data Export

Monthly billable data captured, annual projection, RTM-eligibility rate, full CPT code eligibility table with the patient-app data source for each code, and a stacked monthly export trend chart by code category. This is the view BayMark's billing team uses to convert engagement into revenue. The AI Revenue Optimization card identifies patients below billing thresholds and projects the revenue recovery if engagement is nudged upward — giving clinical operations a direct ROI lever they can act on.

Leadership ROI Dashboard

Annual billable data captured, platform ROI multiple, reduced readmissions, patient retention lift, and a Pre-VictoryAI vs. With-VictoryAI revenue comparison across all code categories. The AI Leadership Summary translates the operational metrics into the language Jason Kletter and the Webster Capital operating committee will use to evaluate enterprise rollout. This is the view that closes the conversion from pilot to enterprise contract.

SECTION 05

Engagement → Billable Code Mapping

This is the section that BayMark's CFO will read first. The promise of VictoryAI is that engagement is not just a clinical good — it is a revenue line. The table below shows exactly which patient-app actions generate documentation for which billing codes, and what data threshold the EMR needs to see before the code becomes claim-ready.

Code	Description	App Action That Generates It	Threshold
98975	RTM Device Setup	Initial app enrollment + first MAT log	Once per patient
98977	RTM Device Supply	Daily app engagement	16+ days/month
98980	RTM Mgmt (first 20 min)	Check-ins + MAT data review	20 min/month
98981	RTM Mgmt (add'l 20 min)	Extended engagement data	Each add'l 20 min
H2019	Digital CBT Sessions	DTx module completion	Per 15-min session
G2213	OUD Treatment Initiation	First-month MAT enrollment + adherence	First month
G2214	OUD Treatment Continuation	Ongoing MAT + engagement	Each subsequent month
99490	CCM Care Management	Between-visit app engagement time	20 min/month
99424	PCM Principal Care	OUD-only patient app time	30 min/month
99495	TCM Moderate	Post-discharge tracking + 2-day contact	$201/episode
99496	TCM High Complexity	TCM checklist + face-to-face	$273/episode
H0005	Group Therapy	App group session check-in	Per session
G0480-83	Drug Screen Toxicology	UDS compliance tracking	Per screen
G2216	Take-Home Naloxone	Narcan hub dispensing record	Per dispensing
99408/09	SBIRT Screening	DAST-10 risk screening	Annual or transition
99484	BHI Care Management	Medication adherence + mood data	Monthly

Code	Description	App Action That Generates It	Threshold
99492/93	CoCM Care Management	BH + PCP collaboration data	Monthly
96156/58	Health Behavior	SDOH + CBT session data	Per intervention
Z59.0+	SDOH Z-codes	Monthly SDOH screener	Each screening

Why This Matters for the Pilot

BayMark's California BAART clinics currently capture methadone dosing, counseling sessions, and standard billing. The codes in the table above — particularly the RTM family (98975–98981), the DTx code (H2019), the OUD treatment codes (G2213/G2214), and the TCM codes (99495/99496) — are largely uncaptured because they require structured patient-engagement data that clinicians do not have time to generate manually. VictoryAI generates that data as a byproduct of the patient using the app for their own recovery.

> *At $490 to $737 in identifiable billing data per OUD patient per month, a 200-patient pilot across 3 to 5 BAART California sites would surface roughly $107,000 per month in previously uncaptured billable data. Over a 12-month enterprise rollout, that is $1.28 million just from California.*

The Leadership ROI Dashboard surfaces this in real time. Every billable event generated by the engagement system is counted, attributed to the originating patient action, and rolled up into the monthly EMR export trend. BayMark's billing team sees what came in. The CFO sees the trajectory. The operating committee sees the path to enterprise rollout.

SECTION 06

The AI Predictive Layer

The predictive layer is where engagement data becomes clinical foresight. VictoryAI's AI predictive analytics combine signals from across the event taxonomy to generate per-patient relapse risk scores, identify pre-relapse trajectories before they become crises, and surface the specific patients who need clinical intervention this week.

The Strongest Predictor: Voice Journal Sentiment

Across the platform's predictive modeling, the strongest single signal for an impending relapse event is a sustained sentiment decline in voice journal entries. NLP analysis of voice journal content — not the raw audio, which is never transmitted — detects mood deterioration five to seven days before self-reported relapse events on average. This signal alone, combined with MAT adherence data, gives BayMark clinicians a window of clinical action that is unavailable in any current OUD treatment workflow.

The Six Contributing Factors

Each individual relapse risk score is computed from six contributing factor categories. The dashboard surfaces both the composite score and the per-factor breakdown, so a clinician can see not just that a patient is at elevated risk but specifically why.

- MAT Adherence Trajectory — recent adherence streak, missed dose pattern, side effect frequency
- Engagement Pattern — Recovery Flame status, daily active days, feature usage breadth, completeness score trajectory
- Family Reward Activity — pledges active, reward claim velocity, gratitude gift reciprocity
- Craving Profile — average craving score, urge mode activation frequency, peak window stability
- Voice Journal Sentiment — sentiment trajectory, decline events, NLP-detected risk language
- Pain-Craving Correlation — pain score trajectory, non-opioid tool usage, correlation coefficient

Population-Level Output

At the population level, the AI Predictive Analytics view surfaces a Risk Score Distribution chart segmenting all enrolled patients into Low (0–30), Medium (31–60), High (61–80), and Critical (81+) buckets. Movement between buckets is itself a tracked event — when a patient moves

from Medium to High, the dashboard fires an alert. When a patient moves from High to Low, the dashboard records the intervention pattern that drove the improvement, contributing to the platform's longitudinal learning.

The predictive layer is the reason VictoryAI is not interchangeable with a wellness app or a generic patient-engagement tool. The engagement events are the input; the predictive output is what makes BayMark's clinicians more effective on Monday morning than they were the previous Friday.

SECTION 07

Pilot Success Metrics

Every pilot needs pre-agreed success criteria that cannot be redefined after the data comes in. The five metrics below are the criteria VictoryAI proposes for the BayMark joint innovation pilot. They are observable in the dashboard from day one, they map directly to the engagement-as-data thesis, and they translate cleanly into the enterprise rollout business case.

Metric	Threshold	Measured From	Why It Matters
Patient Engagement	≥60% active 16+ days/month	Engagement & Gamification view	RTM threshold for 98977 billing
MAT Adherence Lift	≥15% over BayMark baseline	MAT Adherence Analytics view	Direct retention-to-revenue lever
Billable Data Generated	≥$300/patient/month	EMR Data Export view	Pilot self-funds before enterprise rollout
Clinician Adoption	≥80% dashboard usage	Provider analytics	Confirms workflow fit
Technical Reliability	99.5% uptime, zero HIPAA incidents	Platform monitoring	Required for enterprise contract

Reporting Cadence

VictoryAI provides BayMark with three reporting cadences during the ninety-day pilot. A real-time dashboard, available continuously to all credentialed BayMark users. A weekly executive summary delivered to the Operations VP and Clinical Director, surfacing the week's headline metrics and any emerging risks. And a monthly outcomes report delivered to Jason Kletter's leadership team, formatted to support both the Webster Capital operating committee and any DHCS-facing reporting.

At pilot end, VictoryAI delivers a comprehensive ninety-day outcomes report that is publication-ready for ASAM or AAAP conference submission, formatted for DHCS quality reporting, and structured to serve as the business case for enterprise rollout to BayMark's full 300-clinic footprint.

SECTION 08

BayMark Pilot Implementation

This section translates the architecture documented above into a concrete deployment plan for the BayMark joint innovation pilot. The plan assumes a ninety-day active pilot phase across three to five California BAART clinics with a target enrollment of two hundred or more patients.

Pre-Launch (Weeks 1–4)

- Kipu EMR integration sandbox configured and tested against BAART California data flows
- HIPAA Security Risk Assessment delivered, BAA executed, 42 CFR Part 2 segmentation verified
- BayMark branding applied to patient app and provider dashboard (per VictoryAI brand system, co-branded with BayMark/BAART marks)
- Clinician training curriculum delivered to BAART California clinical leads
- Site readiness checklist completed at each pilot clinic (devices, network, posters, consent forms)
- Patient onboarding toolkit deployed with 42 CFR Part 2 consent flow

Launch (Weeks 5–6)

- Phased patient enrollment beginning with the highest-engagement clinic
- Daily VictoryAI customer success check-ins with each pilot site
- Real-time provider dashboard goes live to all credentialed BayMark users
- First weekly executive summary delivered to BayMark leadership

Active Pilot (Weeks 7–17)

- Continuous patient app usage with full event taxonomy firing into the dashboard
- Weekly pulse meetings between VictoryAI customer success and BAART clinical leads
- Monthly outcomes report to Jason Kletter's leadership team
- AI predictive layer activated by week eight, once enough patient data accumulates for individual model training
- Family Reward Board onboarding push during week six and again at week ten — this is the single highest-leverage intervention for retention lift

Pilot Close (Weeks 18–20)

- Comprehensive 90-day outcomes report delivered

- Publication-ready data package for ASAM or AAAP submission
- Enterprise rollout business case prepared for Webster Capital operating committee
- Joint press release executed (held until BayMark approves)
- Enterprise license negotiation begins with pilot fee credit applied per the enterprise credit clause

CLOSING

Why This Document Exists

BayMark has been pitched by digital health companies before. Most of those pitches hand-wave at engagement, hand-wave at outcomes, and hand-wave at ROI. This document exists because VictoryAI does not hand-wave. The behavioral architecture is documented, the event taxonomy is enumerated, the dashboard views are inventoried, the billing code mapping is specific, and the predictive layer is explainable.

The thesis is one sentence long. Engagement is the production line for analytics. Every feature in the patient app is a data event waiting to fire. Every gamification mechanic is a behavioral lever pulling patients toward actions that generate clinical value. Every dashboard view translates that data into a decision a BayMark clinician, operator, or executive can act on this morning.

> *If VictoryAI works, the BayMark pilot will not be remembered as a software deployment. It will be remembered as the moment OUD treatment got an analytics layer.*

The next step is the joint innovation pilot itself: three to five California BAART clinics, two hundred or more patients, ninety days of active deployment, and a comprehensive outcomes report at the end. The architecture is ready. The team is ready. The remaining question is whether BayMark wants to be the lighthouse customer that proves this category exists — or whether one of BayMark's competitors gets there first.

VictoryAI is built for BayMark. This whitepaper is the proof.

VictoryAI · OUD Recovery Companion & Outcomes Analytics Platform

www.getvictoryai.com · Confidential — Prepared for BayMark Health Services · April 2026

Address this:

One thing I'd flag: a few of the precise statistics in here (the 4.2× retention lift, the 22% lift from variable rewards, the 14% Level 2 conversion) came from your existing feature documentation. Before this leaves your hands to BayMark, make sure those numbers are either internally validated or footnoted as projections — BayMark's clinical team will ask where they come from.